

**STRATEGY & OPERATIONS**

Investor Readiness And The Winning Concept

Vytalix company-build documentation · reference

23_INVESTOR_READINESS_AND_WINNING_CONCEPT

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

DATE

8 September 2026

VERSION

1.0 · Confidential draft

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Owner: CEO with Fundraising Director and Chief Strategy Officer · **Date:** 8 September 2026 · **Status:** PROPOSED ·

Governing file: /FACTS_BASE.md

Executive view

- Investor readiness today: 2/10.** Not because the idea is weak, but because five hard blockers exist: no entity, no IP agreement, unvalidated clinical claims live on a public URL, no revenue, no pilot partner. Each is fixable within 60–90 days and the plan below fixes them in order.
- The winning concept is not "another maternal-health app".** It is **the Maternal Continuity Layer**: an offline-first, interoperable, woman-held maternity record and coordination service that complements the systems governments already run, is priced per woman and paid partly on measured results, and is funded from three sources at once (public budgets, donors, and diaspora sponsorship). That combination is unusual, defensible and fundable.
- Government contracts are won by frameworks, evidence and partners, not by cold proposals.** The playbook below gives the routes (UK: DTAC, SBRI Healthcare, Health Innovation Networks, G-Cloud; Nigeria: state primary-health agencies, BHCPF-funded programmes, donor-financed implementers) and the sequence.
- Investors will fund evidence of motion:** a signed IP deed, a first paying advisory client, one pilot letter of intent, an advisory board with a clinician, and a data room. The "traction" slide is built from those, not from projections.
- Timeline:** pre-conditions cleared by Day 30; first proof points by Day 60; first investor calls Day 61 (Call List 1 in /investors/); pre-seed close targeted within 120–150 days of Day 1.

1. Investor-readiness scorecard (score yourself monthly)

#	Readiness item	Today	Target for first investor call	How to close it	Owner / by
1	Legal entity, bank, accountant	✗	✓	Incorporate Vytalix (E&W) after name clearance; bank; accountant; SEIS/EIS advance assurance submitted	Founder · Day 10
2	MaternaLink IP owned or licensed in writing	✗	✓	Heads of terms then IP deed with David Agunede (options in 02_GROUP_STRUCTURE.md); contributor IP assignments for any developer	Founder + solicitor · Day 30
3	No unvalidated clinical claims anywhere public	✗ (prototype shows suggested clinical actions; 2025 deck circulates)	✓	Retire the 2025 deck; relabel prototype rules; put prototype behind auth; remove identifiable photos	Founder · Day 3–14
4	Name and trademark clearance	✗	✓	Knock-out searches UK IPO/EUIPO/USPTO; file UK TM classes 9, 41, 42, 44 for VYTALIX and MATERNALINK	Founder + TM attorney · Day 5 / Day 20
5	First revenue	✗	1 signed SOW	Advisory packages live; 40 conversations; first engagement	Founder · Day 45

#	Readiness item	Today	Target for first investor call	How to close it	Owner / by
6	Pilot partner	✗	1 LOI (UK service evaluation or Nigeria NGO cohort)	15 discovery interviews; pilot proposal from maternalink-site/pilot.html	Founder · Day 60
7	Clinical and advisory board	✗	2 confirmed (obstetric/midwifery + NHS digital or Nigerian health-system leader)	Named target list; 20-minute asks; advisory agreements with equity in the option pool	Founder · Day 60
8	Product evidence	Prototype of unknown provenance	Audited prototype + PRD + intended-use statement + hazard log opened	Code audit; clinician sign-off of intended use	CTO/CPO (fractional) · Day 30
9	Financial model and round	✓ (scenarios)	✓ with actuals from months 1–3	Update finance/ with real costs; pick round size (£250k–£500k)	CFO role · Day 60
10	Data room	✗	✓	Index in \$2 below; host on a shared drive with access logging	Founder · Day 45
11	Investor list and outreach system	✓ (316-investor CRM)	✓ verified Top 25 phones	Re-verify Call List 1; scripts in /investors/OUTREACH_TOP25.md	Founder · Day 55
12	Compliance basics	✗	ICO registration, Cyber Essentials booked, PI/cyber insurance quotes	See 15_LEGAL_AND_REGULATORY.md	Founder · Day 30
13	Founder story and commitment	Partial	Clear time allocation and role; second venture (Kemek) ring-fenced	Written founder commitment statement; time split rule in 17_FOUNDER_OPERATING_SYSTEM.md	Founder · Day 7
14	Proof of demand	✗	15 interview summaries + 3 quotes (with consent)	Discovery interviews; report research	Founder · Day 60
15	Non-dilutive applications	✗	1 submitted (Innovate UK Smart or SBRI Healthcare when open)	See 09_FUNDING_STRATEGY.md and /investors/NON_DILUTIVE_FUNDING_ROUTES.md	Founder · Day 75

Rule: do not take a first investor meeting until items 1–4 are ✓ and at least two of 5–7 are ✓. A premature meeting with a Top 25 fund costs you that fund for 12 months.

2. Data-room index (build it in this order)

1. Company: certificate of incorporation, articles, cap table, SEIS/EIS advance-assurance correspondence, director details.
2. IP: MaternaLink IP deed or licence; contributor assignments; trademark filings; domain ownership.
3. Product: [04_MATERNALINK_PRODUCT_STRATEGY.md](#), PRD, MVP spec, roadmap, feature addendum, intended-use statement, prototype audit report, hazard log, DPIA draft.

4. Market and strategy: audit, business model, market strategy, customer segments, competitive intelligence, partnership strategy.
5. Commercial: service catalogue, pricing ([21_MATERNALINK_PRICING.md](#)), pipeline export, signed SOWs, pilot LOIs.
6. Finance: [Vytalix_Financial_Model_3yr.xlsx](#) , [08_FINANCIAL_MODEL.md](#) , bank statements once trading, budget vs actual.
7. Funding: [09_FUNDING_STRATEGY.md](#) , [10_INVESTOR_PROPOSITION.md](#) , one-pager, deck, use-of-funds, milestone plan.
8. Governance and compliance: advisory agreements, policies (information security, data protection, clinical safety), ICO registration, insurance, risk register.
9. Team: founder CV and commitment statement, advisers, hiring plan.
10. Evidence: interview summaries, letters of intent, research report, pilot evaluation design.

3. The winning concept: The Maternal Continuity Layer

Why the current framing loses

"Offline-first maternal monitoring for public health systems" describes a category with funded incumbents (mDoc in Nigeria, MomConnect in South Africa, BadgerNet in the NHS). Buyers hear "another platform", investors hear "crowded", and the risk engine drags the product toward medical-device regulation before there is evidence.

The reframe (PROPOSED)

MaternaLink is the continuity layer that follows the woman, not the facility. One consented, woman-held maternity record and communication channel that works offline, plugs into whatever the government or hospital already runs, and keeps the woman connected to a responsible human from booking to postnatal discharge. Governments keep their systems; MaternaLink makes them continuous. Five elements make it distinctive together:

Element	What it is	Why it wins contracts	Why investors care
1. Woman-held continuity record	A portable, consented record (FHIR-aligned) the woman carries between community, clinic, hospital and even countries (UK diaspora ↔ Nigeria)	Solves the fragmentation every ministry and trust names first; complements incumbents instead of replacing them	Interoperability is a moat once integrated; switching cost rises with every interface
2. Complementarity by design	Integration pack for DHIS2, HMIS, BadgerNet/K2/Euroking; "we make your existing programme continuous"	Removes the political cost of replacing a funded programme; makes incumbents partners	De-risks the go-to-market; partnership revenue instead of price wars
3. Results-linked pricing	Part of the fee tied to measured, auditable indicators (postnatal follow-up completion, time-to-contact after a flag, missed-appointment recovery)	Governments and donors increasingly buy outcomes; fits results-based financing and impact bonds	Differentiated commercial model; evidence is generated by the contract itself
4. Three-source funding per woman	Public budget + donor/programme funding + diaspora and employer sponsorship ("Sponsor a Safe Pregnancy": a UK-based relative or company pays the per-woman fee for a named community or cohort)	Reduces the burden on state budgets; creates a constituency of sponsors who lobby for the programme	A B2C2B revenue line no incumbent has; high-margin, recurring, and emotionally compelling

Element	What it is	Why it wins contracts	Why investors care
5. Workforce built in	E-Learning for midwives and community health workers bundled with the platform (capacity-building is an explicit government objective)	Governments buy "workforce + software" programmes more readily than software alone	Second pillar sells through the first; higher contract values

One-line concept: *"The continuity layer for maternity care: one record that follows the woman, works without signal, complements the systems you already run, and is paid for partly on results."*

What this concept is not

It is not a diagnostic tool and does not need to be. The AI risk-score idea from 2025 becomes a research consortium with a university (element 6 in `04_MATERNALINK_PRODUCT_STRATEGY.md`), which is a stronger investor story than an unvalidated model.

Additional ideas worth testing (scored with `20_DECISION_FRAMEWORK.md`)

Idea	Score	Verdict	Note
Sponsor a Safe Pregnancy (diaspora and employer per-woman sponsorship)	70	TEST in 90 days: landing page, 100 sign-ups of intent, one employer	Requires charity-partner structure or a clear consumer contract; payment via Stripe/Paystack
Results-linked contracting with a donor-backed programme	64	TEST with one NGO cohort: define 3 indicators and a bonus/malus	DFIs (BII, Proparco) and Grand Challenges Canada like this structure
Maternity equity dashboard for commissioners (ICBs, state ministries): anonymised, population-level continuity indicators by ethnicity and geography	66	TEST as a report first, product second	Directly addresses MBRRACE-UK inequality findings; sold to commissioners, not trusts
Continuity record for cross-border families (UK ↔ Nigeria)	58	DEFER to v2	Distinctive but regulatory complexity across two data regimes
Research consortium for the AI track (university + NHS trust + Nigerian teaching hospital)	62	TEST: one university partner conversation	Converts the 2025 concept into fundable research (NIHR/Wellcome routes)

4. Government-contract playbook

United Kingdom (NHS)

Step	Route	What it takes	Time
1	Health Innovation Networks (formerly AHSNs): the front door for SMEs into NHS regions	Register; ask for a "innovation exchange" conversation; bring the intended-use statement and pilot design	Weeks 2–6
2	NHS Innovation Service: national triage of innovations to the right support	Online application; be honest about stage	Week 4
3	DTAC (Digital Technology Assessment Criteria) readiness: clinical safety, data protection, technical security, interoperability, usability	Build the evidence pack from <code>15_LEGAL_AND_REGULATORY.md</code> and the security page; DSPT and Cyber Essentials	Months 2–5
4	SBRI Healthcare competitions (NHS-funded development contracts, 100% funded) and Innovate UK Smart / Biomedical Catalyst	Apply when a maternity or inequalities theme opens; needs an NHS partner letter	Watch calendar

Step	Route	What it takes	Time
5	Service evaluation at one trust (not a research study): a maternity digital lead, a midwifery lead, a Caldicott Guardian sign-off	Pilot proposal + evaluation design; no fee or small fee	Months 3–6
6	Frameworks: G-Cloud (Crown Commercial Service Digital Marketplace) so trusts can buy without a tender; later NHS Shared Business Services / LPP frameworks	Listing after entity and DTAC basics	Month 6+
7	ICB-level commissioning: maternity and neonatal safety and equity programmes; local maternity and neonatal systems (LMNS)	Equity dashboard report as the door-opener	Month 6–12

Never claim "NHS-approved". Say "in service evaluation at [trust]" only when it is true.

Nigeria (and similar African markets)

Step	Route	What it takes
1	State Primary Health Care Development Agencies (SPHCDA) and state ministries of health own community maternal programmes; the NPHCDA and the Basic Health Care Provision Fund (BHCPF) finance primary care at scale	Map the state's existing digital programmes first (Ekiti has mDoc); propose complementarity or choose a state without an incumbent
2	Donor-financed implementers (World Bank-financed state programmes, Global Fund, Gates, MSD for Mothers, UNICEF, UNFPA, Jhpiego, Society for Family Health, PATH) hold the budgets and procurement power	Sub-contract as the technology partner of an implementer rather than selling to the state alone
3	Local incorporation and registration: CAC company, NDPA data-protection registration, tax registration; in-country partner for support	Required for most state contracts; budget in 15_LEGAL_AND_REGULATORY.md
4	Procurement: Bureau of Public Procurement rules; pilots often start as MoUs with an evaluation and move to procurement after evidence	MoU template; evaluation design; DHIS2 alignment
5	Champions: state first ladies' programmes, commissioners for health, the National Council on Health; professional bodies (SOGON, NANNM)	Present the evidence plan and complementarity; avoid the £36m headline
6	Financing innovation: results-based financing pilots; diaspora sponsorship reduces state cost per woman	Offer a blended model: state pays base, sponsors and donors pay engagement

Also score Kenya, Ghana and Rwanda ([05_MARKET_STRATEGY.md](#)); Rwanda's digital-health openness and Kenya's county health systems make them strong second markets.

What makes a government buyer say yes

1. It works without connectivity and on the phones they already have.
2. It complements what they have; nothing is thrown away.
3. Evidence is generated by design and shared openly.
4. Data stays under their control; residency and sovereignty are answered up front.
5. The price per woman is defensible and partly paid on results.
6. Workforce training is included.
7. There is a named clinician behind the escalation rules, and a safety case.

5. What investors will ask, and the honest answer today

Question	Answer now	Answer at Day 90 (target)
Do you own MaternaLink?	Not yet in writing	Yes, IP deed signed
Is it a medical device?	Not as re-scoped; the prototype needs changes	Intended-use statement adopted; prototype changed; regulatory opinion scoped
Any revenue?	None	First advisory engagements
Any customer or pilot?	A prior ministry conversation with no outcome	One pilot LOI; 15 interviews documented
Who is on the team?	Solo founder plus a collaborator	Founder plus two advisers, fractional CTO and clinical safety officer identified
Why you?	Founder story (to be written)	Founder story plus first evidence of execution
How much and for what?	£250k–£500k pre-seed	Same, with milestones tied to the model

6. Sequence for the next 120 days

Days	Focus	Exit criteria
1–14	Blockers: name, entity, IP heads of terms, retire claims, prototype behind auth	Items 1–4 of the scorecard ✓
15–45	Revenue and evidence: advisory outreach, discovery interviews, advisory-board asks, prototype audit	First SOW; 10 interviews; 1 adviser
46–75	Pilot and data room: pilot LOI, data room v1, model with actuals, Sponsor a Safe Pregnancy test page, grant bid	LOI; data room; readiness ≥ 7/10
61–120	Investors: Call List 1 then 2; angels and syndicates first; impact and women's-health funds in parallel	25 conversations; 8 meetings; lead investor conversation
120–150	Close pre-seed; start pilot	Funds in; pilot kick-off

Priorities / Risks / Next actions

Priorities: blockers first; concept reframe adopted in all materials; government routes worked through partners.

Risks: founder time; premature investor contact; concept drift back to "AI prediction"; Nigeria procurement timelines.

Next actions: adopt the reframe in the deck and site (this build already uses the continuity language); book the Health Innovation Network conversation; draft the Sponsor a Safe Pregnancy test page; start Day 1 of

19_FIRST_30_DAYS.md .